

AND OTHER EMERGENCIES

Week 1: 29 December to 4 January 2026
Data as reported by: 17:00; 4 January 2026



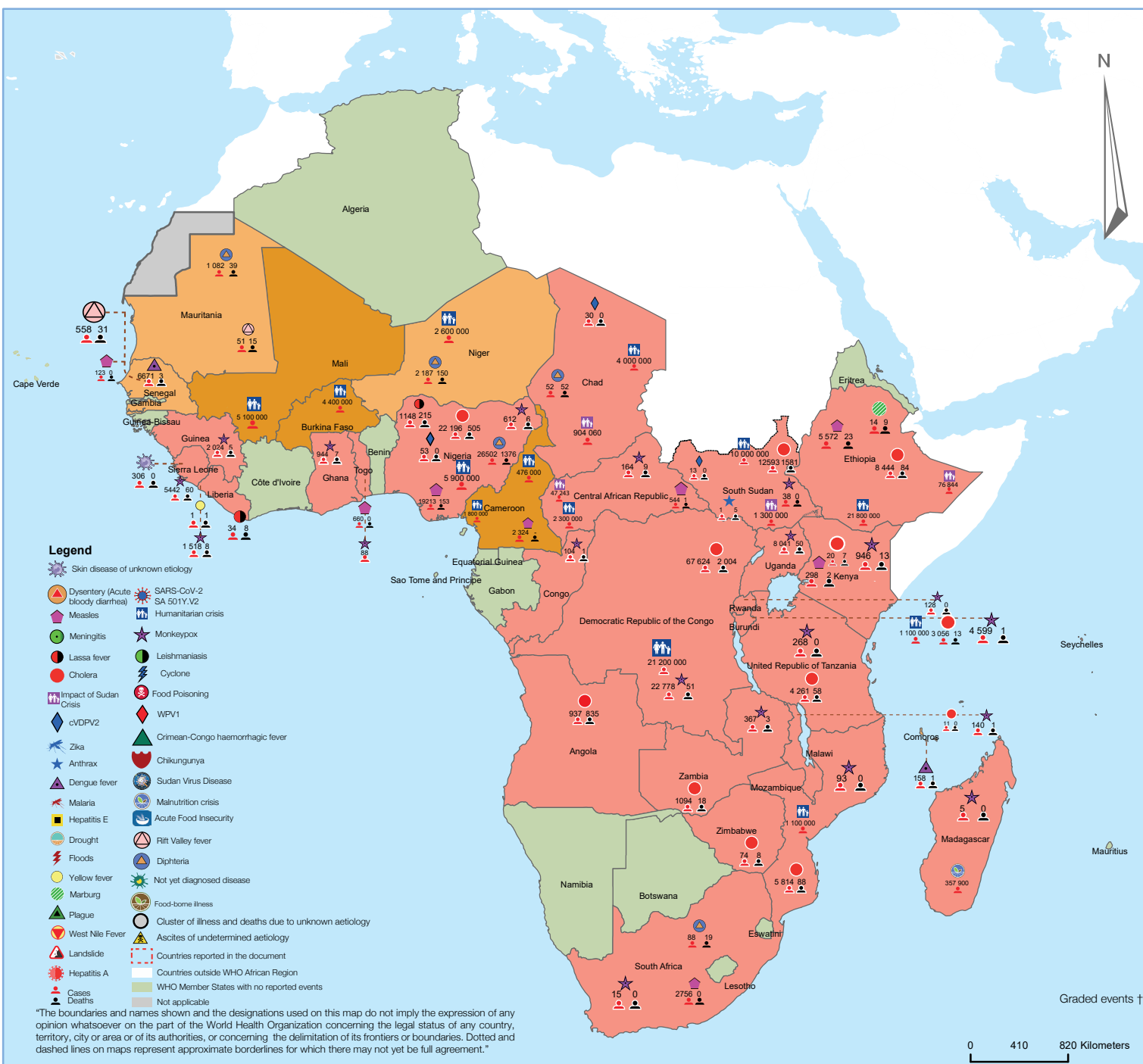
African Region

New events

Ongoing events

Outbreaks

Humanitarian crises



“The boundaries and names shown and the designations used on this map do not imply the expression of any opinion whatsoever on the part of the World Health Organization concerning the legal status of any country, territory, city or area or of its authorities, or concerning the delimitation of its frontiers or boundaries. Dotted and dashed lines on maps represent approximate borderlines for which there may not yet be full agreement.”

Grade 3 events

Grade 2 events

Grade 1 events

Ungraded events

Protracted 3 events

Protracted 2 events

Protracted 1 events

Overview

Contents

1 Overview

2 - 9 Ongoing events

10 All events currently being monitored

This Weekly Bulletin focuses on public health emergencies occurring in the WHO African Region. This week's articles cover:

- Rift valley fever in Senegal
- Rift valley fever in Mauritania
- Humanitarian crisis in the Democratic Republic of the Congo

For each of these events, a brief description, followed by public health measures implemented and an interpretation of the situation, is provided.

A table is provided at the end of the bulletin with information on all new and ongoing public health events currently being monitored in the region, as well as recent events that have been controlled and closed.

Major issues and challenges include

- **Rift Valley fever in Senegal and Mauritania:** The Rift Valley fever outbreaks in Senegal and Mauritania highlight persistent vulnerabilities at the human–animal–environment interface, driven by seasonal ecological conditions, widespread livestock mobility, and close human–animal contact in pastoral settings, which have facilitated zoonotic spillover and rapid geographic spread. Although the outbreaks appear under control in both countries, ongoing animal infections, cross-border movements, and limited surveillance and response capacity in high-risk rural areas continue to pose a risk of resurgence. Sustained One Health and cross-border surveillance, targeted livestock vaccination, and continued risk communication with at-risk communities are essential to consolidate gains, prevent re-emergence, and mitigate future outbreaks.
- **Humanitarian Crisis in the Democratic Republic of the Congo:** The crisis in eastern DRC is marked by ongoing insecurity, large-scale displacement, and severe constraints on humanitarian access, resulting in major disruptions to health services and increasing the risk of outbreaks, malnutrition, and excess mortality, with significant spillover into neighbouring countries. These challenges are unfolding amid a global humanitarian funding crisis that is constraining the scale and continuity of health and humanitarian responses in affected and host communities. Addressing the situation requires urgent protection of civilians and health infrastructure and a cessation of hostilities to enable sustained humanitarian access, alongside immediate and increased funding to adequately resource response activities to prevent further deterioration of health outcomes.

Senegal

561
Cases

31
Deaths

5.5%
CFR

Rift valley fever

EVENT DESCRIPTION

The outbreak of Rift Valley fever (RVF) in Senegal appears to be under control, with no new human cases reported in the two weeks leading up to 4 January 2025. The most recent confirmed case had symptom onset on 14 December 2025 and was reported from Podor district in the Saint-Louis Region. The current outbreak was first reported in late September, when the first two human cases were detected on 20 September 2025 in Saint-Louis district, Saint-Louis Region, before subsequently spreading to other parts of the country.

From 20 September 2025 to 4 January 2026, a total of 561 confirmed human RVF cases, including 31 deaths (CFR 5.5%), were reported from 41 districts across 11 regions of the country. Saint-Louis Region was the most affected, accounting for 66.5% (n=373) of cases and 61.3% (n=19) of deaths. The remaining affected regions were Matam (46 cases, 5 deaths), Fatick (39 cases, 3 deaths), Kaolack (37 cases, 0 deaths), Louga (22 cases, 4 deaths), Dakar (18 cases, 0 deaths), Tambacounda (10 cases, 0 deaths), Kolda (5 cases, 0 deaths), Thiès (5 cases, 0 deaths), Kaffrine (2 cases, 0 deaths), and Kédougou (2 cases, 0 deaths).

Cases ranged in age from 1 to 94 years, with a median age of 27 years. The majority of cases occurred among individuals aged 15 to 35 years, accounting for 59.0% (n=331) of cases and 71.0% (n=22) of deaths. Males were disproportionately affected, representing 64.9% (n=364) of cases. By occupation, cattle herders, students, and housewives are the most affected groups. Haemorrhagic symptoms were reported in 46 cases, including 23 of the 31 reported deaths.

Concurrently, confirmed Rift Valley fever cases have been reported among animals. As of 4 January 2026, a total of 446 animal cases, primarily affecting sheep, goats, and camels, have been reported across 11 regions of the country. The affected regions include Saint-Louis, Louga, Matam, Kolda, Fatick, Tambacounda, Dakar, Thiès, Diourbel, Kaffrine, and Kaolack. In addition, samples from 103 animal abortions have been collected and submitted for laboratory investigation.

The previous outbreak of human Rift Valley fever in Senegal was reported in January 2025 in Touba, Diourbel Region.

PUBLIC HEALTH ACTIONS

- Incident Management Systems have been activated at both national and regional levels in accordance with the national contingency plan. The Zoonoses Thematic Group under the National Health Security Council (One Health platform) remains actively engaged, ensuring coordination between the human, animal, and environmental health sectors.
- Incident Action Plan 3 was finalized to guide ongoing response activities, and supervisory missions were conducted in Diourbel Region to support implementation and coordination at subnational level.
- Enhanced surveillance activities, including active case finding and retrospective record reviews, are ongoing in affected and high-risk areas. Multidisciplinary SURGE teams were deployed to Saint-Louis, Louga, Matam, and Fatick Regions to strengthen early detection, reporting, and epidemiological investigation.
- Surveillance of animal health events, including abortions and unexplained livestock deaths, continues to inform risk assessment and response planning.
- Case investigations and systematic sample collection from suspected human and animal cases are ongoing. Mobile laboratory capacity has been deployed to support timely diagnostic testing, reduce turnaround times, and strengthen confirmation of cases, particularly in remote or high-burden areas.
- Samples from animal abortions are being prioritized for laboratory analysis to better characterize ongoing transmission dynamics.
- Suspected and confirmed severe RVF cases are being managed in designated health facilities in line with national clinical management guidelines.
- Health workers have been sensitized on early recognition of severe disease manifestations, including haemorrhagic

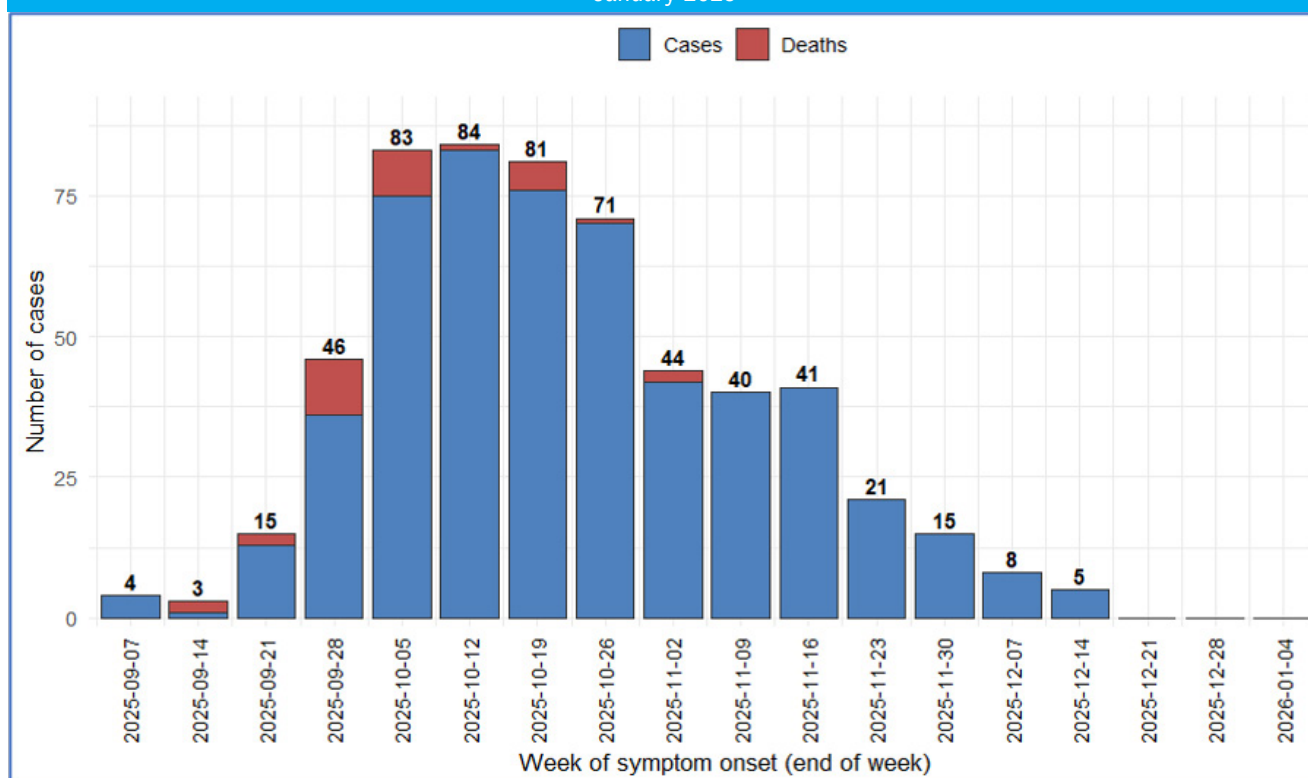
symptoms, and on appropriate referral pathways to reduce morbidity and mortality.

- Infection prevention and control measures have been reinforced in health-care settings, including standard precautions, safe handling of biological samples, and appropriate waste management.
- IPC guidance has been shared with health facilities in affected regions to minimize the risk of nosocomial transmission.
- Risk communication and community engagement activities have been intensified to promote early care-seeking and reduce exposure to infection. Communication materials were developed and widely disseminated, community radio programmes are airing in affected areas, and door-to-door sensitization activities are ongoing.
- To date, 16 265 herders have been sensitized on RVF prevention measures, including safe animal handling, reporting of animal abortions, and avoidance of consumption of sick or dead animals.
- Operational support continues to facilitate response activities across sectors. Perifocal and targeted vaccination of unaffected livestock is ongoing, with 51,901 small ruminants vaccinated to date and approximately 54,000 vaccine doses procured.
- Vector control measures have been implemented in affected areas, including indoor residual spraying, treatment of water bodies, application of acaricides on livestock, spraying of affected households and schools, and distribution of insecticide-treated mosquito nets to reduce human–vector contact.

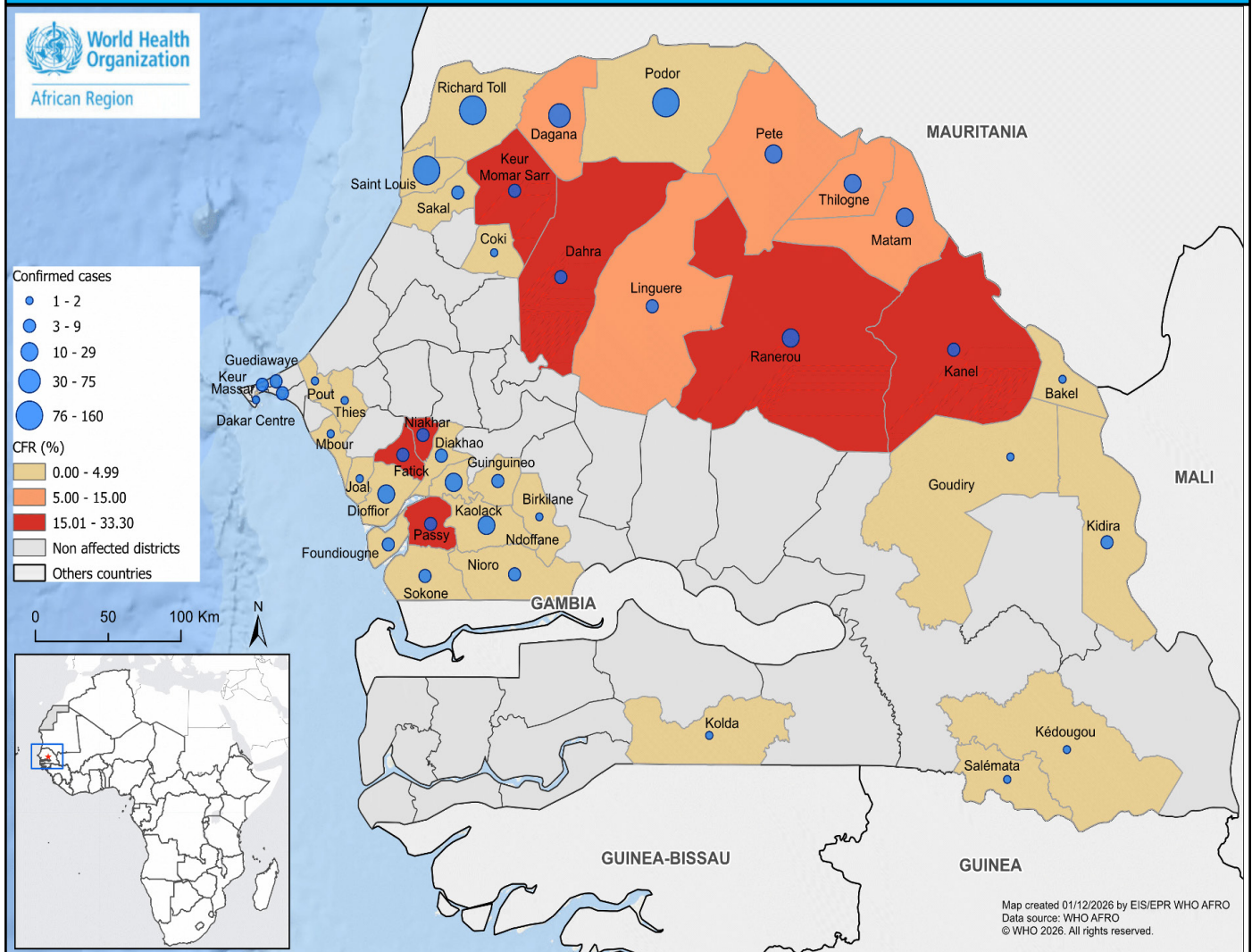
SITUATION INTERPRETATION

The Rift Valley fever outbreak in Senegal has had a substantial public health impact, marked by widespread transmission, a high case burden, and an elevated case fatality ratio. Transmission has been driven by intense exposure at the human–animal–vector interface, with severe disease accounting for most fatalities. Early animal infections preceded and sustained human transmission, underscoring the central role of zoonotic dynamics. Although human cases have recently declined, continued animal infections and ecological risk factors indicate an ongoing risk of resurgence, highlighting the need for sustained One Health surveillance and response.

Confirmed cases and deaths of human Rift Valley fever cases in Senegal by week of symptom onset, 7 September 2025 – 04 January 2026



Geographical distribution of confirmed human Rift Valley fever cases and case fatality ratio by district, Senegal, 20 September 2025 – 04 January 2026



Mauritania

53	15	28.3%
Cases	Deaths	CFR

Rift valley fever

EVENT DESCRIPTION

The outbreak of human RVF in Mauritania, first detected and reported in late September 2025, appears to be under control, with no new confirmed cases reported since epidemiological week 48 of 2025 (week ending 30 November 2025). The most recent confirmed case was reported in epidemiological week 47 (week ending 23 November 2025) from Hodh El Chargui Wilaya (region). The detection of human cases coincided with increased animal infections and seasonal conditions favourable for vector proliferation.

From epidemiological week 39 of 2025 (week ending 28 September 2025) to epidemiological week 1 of 2026 (week ending 4 January 2026), a total of 53 confirmed human cases, including 15 deaths (CFR 28.3%), were reported from 13 wilayas (regions), three of which share international borders: Assaba (bordering Mali to the south) and Brakna and Trarza (both bordering Senegal along the Senegal River). Epidemiological data indicate that case incidence peaked in epidemiological week 40 (week ending 5 October 2025), when 22 confirmed cases were reported.

Concurrently, RVF transmission in Mauritania occurred in the context of widespread animal infections that

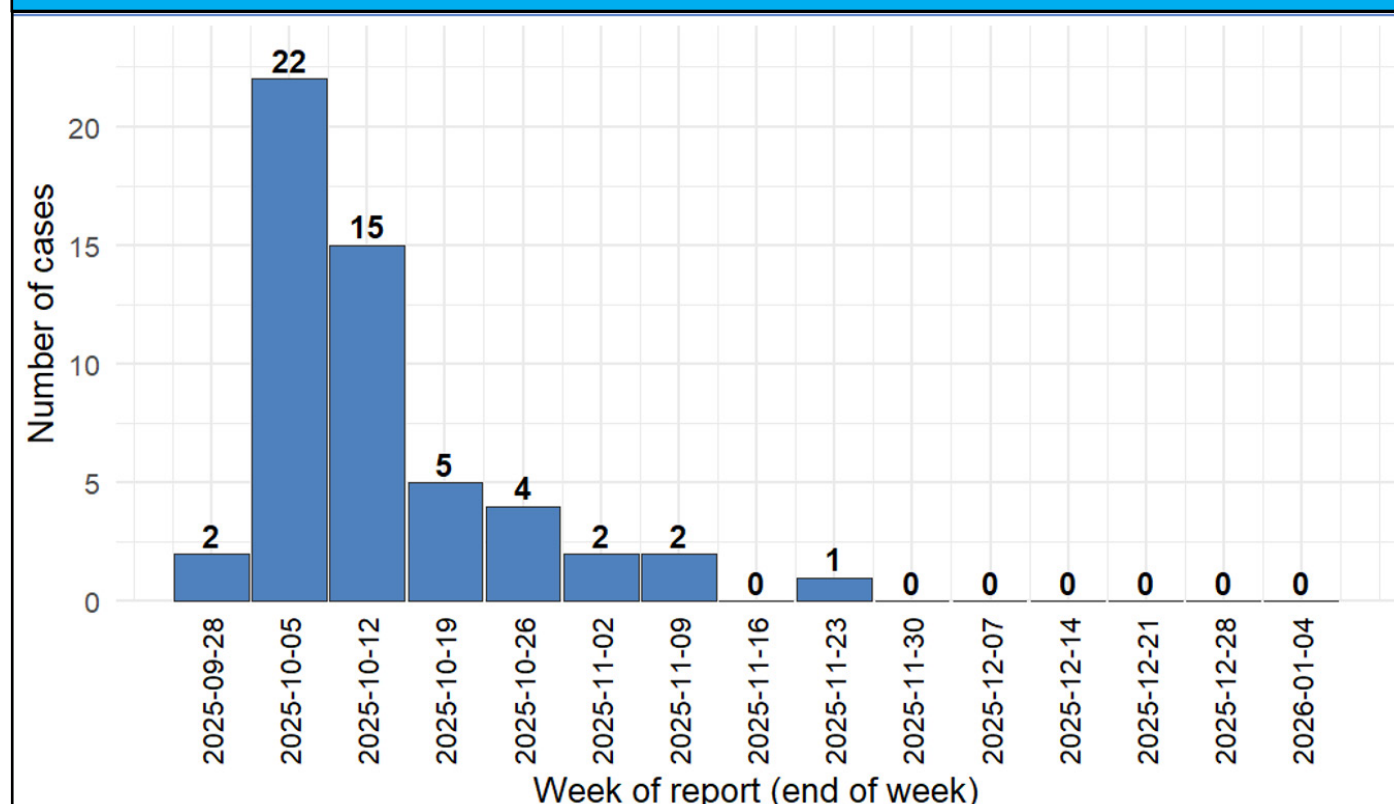
preceded and accompanied human cases. Since August 2025, the Mauritanian Ministry of Animal Resources has reported a total of 68 confirmed animal outbreaks nationwide, with 253 positive samples out of 1,308 collected and tested. Affected species include sheep, goats, dromedaries, and cattle. The first animal cases, involving goats and dromedaries, were reported in August 2025.

Rift Valley fever is endemic in Mauritania. The most recent major outbreak occurred in 2022, during which 47 confirmed human cases, including 23 deaths (case fatality ratio: 49.0%), were reported, primarily affecting pastoralist populations in nine of the fifteen regions.

PUBLIC HEALTH ACTIONS

- Incident management and coordination mechanisms were activated at national and subnational levels to implement the RVF contingency plan and coordinate response pillars.
- A One Health approach was reinforced through joint coordination between the Ministry of Health, Ministry in charge of Livestock and Animal Resources, environmental and vector-control actors, and partners to

Confirmed cases of human Rift Valley fever cases by week of report, Mauritania, 29 November 2025 – 4 January 2026



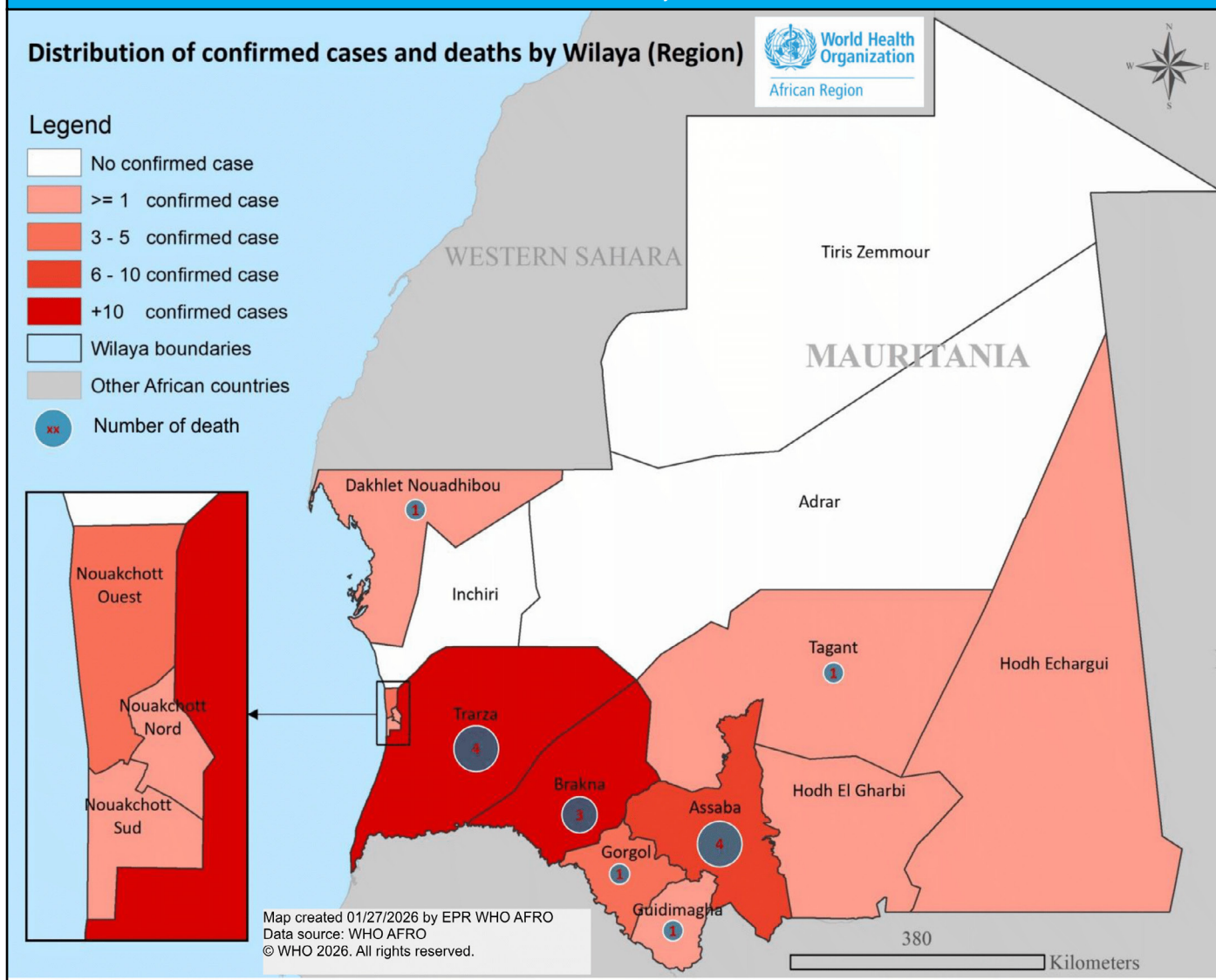
align epidemiological investigations, animal surveillance, and control measures.

- Cross-border coordination was enhanced in wilayas bordering Senegal and Mali given livestock mobility and shared ecological risks, with information-sharing on animal events and human alerts.
- Animal events were routinely reported through the World Organization for Animal Health (WOAH) mechanisms as RVF is a WOAHL-listed disease, supporting coordinated regional situational awareness.
- Surveillance was enhanced in affected and high-risk wilayas, including active case finding, community-based reporting, and retrospective review of health facility records for compatible syndromes (acute febrile illness with haemorrhagic signs, neurologic symptoms, or ocular complications).
- Integrated human – animal surveillance was reinforced, including alerts for livestock abortions and deaths as early signals of increased transmission risk.
- Laboratory confirmation was strengthened through expanded testing capacity and improved sample referral, prioritizing RT-PCR for acute infection and IgM/IgG ELISA depending on timing of illness.
- Quality assurance and biosafety procedures for specimen handling, transport, and testing was reinforced, with coordination between human and animal laboratories to support timely confirmation and joint interpretation.
- Health facilities were sensitized to early identification of severe RVF (including haemorrhagic manifestations), triage, and referral pathways.
- Clinical management focused on supportive care, with close monitoring for complications such as haemorrhagic disease and neurologic/ocular involvement.
- Coordination with blood services and referral hospitals has been strengthened to manage severe bleeding and shock.
- IPC measures in health-care settings have been reinforced, including standard precautions, safe injection practices, safe waste management, and appropriate PPE for clinical care and specimen handling.
- Risk communication was intensified in affected wilayas targeting high-risk groups (herders, farmers, slaughterhouse workers, butchers, veterinarians, and households handling sick animals).
- Community leaders, livestock associations, and market actors are engaged to adapt messages to local practices and to support safe animal handling and reporting.

SITUATION INTERPRETATION

The Rift Valley fever outbreak in Mauritania appears to be under control following a short but intense transmission period that coincided with widespread animal infections and favourable ecological conditions for vector proliferation. The outbreak occurred within a high-risk regional context, characterized by concurrent RVF activity in neighbouring Senegal, extensive cross-border livestock mobility, and recurrent transmission in Mauritania's pastoral zones. The rapid decline in human cases suggests that enhanced multisectoral response measures, together with seasonal transitions, contributed to controlling the outbreak. Continued One Health surveillance and cross-border coordination remain essential to prevent resurgence and mitigate future seasonal re-emergence in this endemic setting.

Geographical distribution of confirmed human Rift Valley fever cases by Wilaya (Region), Mauritania, 28 September 2025 - 4 January 2026



Democratic Republic of the Congo

Humanitarian crisis

EVENT DESCRIPTION

The Democratic Republic of the Congo (DRC) entered January 2026 with a severe and compounding health emergency in its eastern provinces, driven by persistent insecurity, repeated population displacement, and significant constraints on humanitarian access. These shocks continue to disrupt essential primary health care services, including routine immunization and disease surveillance, as well as widespread protection risks, the provision of safe water, sanitation, and hygiene (WASH) services for vulnerable populations, particularly children and displaced persons, resulting in recurrent disease outbreaks and other conditions that increase preventable morbidity and mortality.

Renewed armed clashes in and around Uvira, South Kivu Province, have further aggravated an already dire humanitarian situation, with more than 101 000 people reportedly crossing into neighbouring Burundi since late December 2025. This influx has overwhelmed reception capacities and placed additional pressure on already overstretched health services, further increasing the risk of diarrhoeal diseases, respiratory infections, and acute malnutrition in overcrowded settings. Cholera remains a major concern, with hundreds of cases and scores of deaths reported since December 2025, while measles outbreaks are ongoing among children in the crowded facilities. Other countries hosting refugees include

Uganda, Rwanda, Tanzania, and Zambia.

Within the DRC, ongoing insecurity and access constraints continue to limit outreach, referral, and continuity of care, while the broader national disease burden remains high. In 2025, the DRC experienced an exceptionally high cholera burden, with more than 71 000 cases, including over 2 000 deaths, reported nationwide. Outbreaks of measles, mpox, and other infectious diseases continue to persist.

Nutritional vulnerability has also remained a critical concern as the crisis erodes livelihoods and interrupts health and nutrition services. The Integrated Food Security Phase Classification analysis for July 2025 – June 2026 projects that 4.18 million children aged 6 – 59 months will require treatment for acute malnutrition in DRC (including more than 1.35 million with severe acute malnutrition), alongside approximately 1.54 million pregnant and breastfeeding women expected to need treatment, an outlook consistent with heightened risk of excess mortality in conflict-affected eastern areas

PUBLIC HEALTH ACTIONS

- National and local authorities are coordinating the health response with humanitarian partners through inter-agency mechanisms, prioritizing continuity of essential health services for displaced populations and border-



© UNHCR

area host communities.

- Coordination has been strengthened across health and WASH actors to mitigate outbreak risk in overcrowded sites, where insufficient water and sanitation are driving waterborne disease transmission.
- Health partners are supporting enhanced disease surveillance in reception/transit sites and host communities, with a focus on early detection and reporting of priority epidemic-prone conditions.
- Case definitions, alert verification, and referral pathways are being reinforced to enable rapid investigation and targeted response around suspected outbreak clusters.
- Primary health care and clinical case management are being delivered through fixed facilities and mobile clinics in high-burden sites; in Busuma (Burundi), a single mobile clinic has reportedly been providing very high daily consultation volumes, largely for malaria and acute watery diarrhoea, highlighting the need to expand clinical capacity and referral systems.
- Support is being provided to strengthen triage, referral, and the availability of essential medicines and supplies, including for dehydration management and severe disease.
- Specimen collection and referral systems are being used to support confirmation of suspected outbreaks, while partners continue to advocate for improved diagnostic capacity in overstretched settings.
- Infection prevention and control measures are being reinforced in health facilities and mobile clinics, including standard precautions, safe waste management, and appropriate PPE use, particularly in the context of high patient load and ongoing outbreaks.
- RCCE activities are being implemented with refugee leadership and community structures to promote early care-seeking, safe hygiene practices, and outbreak prevention behaviours, while strengthening accountability and feedback channels.
- Messaging prioritizes cholera prevention (safe water handling, handwashing, prompt treatment for acute watery diarrhoea) and measles awareness.
- Partners are scaling up operational support for frontline delivery, including expansion of mobile services, ambulance and referral capacity, and pre-positioning of essential medical supplies to manage high caseloads and outbreak threats.
- WASH logistics are being prioritized, including water production and storage scale-up, latrine construction, hygiene supplies, and solid waste management

SITUATION INTERPRETATION

The ongoing conflict in eastern DRC is characterized by large-scale displacement with significant regional spillover, placing sustained pressure on health systems in neighbouring countries. The combined disruption of health services and WASH infrastructure, along with restricted humanitarian access within the DRC, and overstretched reception and service capacity in host communities, has strained humanitarian response efforts and exceeded existing response capacity. Without strengthened cross-border coordination, sustained humanitarian access, reinforced health system capacity, and a minimum level of security and operational stability, the regional public health risk is likely to increase further.

All events currently being monitored by WHO AFRO

Country	Event	Grade	Date notified to WCO	Start of reporting period	End of reporting period	Total cases	Cases Confirmed	Deaths	CFR
New Events									
Madagascar	Mpox	Grade 3	29-Dec-25	12-Dec-25	4-Jan-26	5	5	0	0.00%
On 30 December 2025, an mpox outbreak was officially declared in Madagascar. The first suspected case was reported on 12 December 2025 and was laboratory confirmed by PCR on 17 December 2025. As of 4 January 2026, five laboratory-confirmed cases had been reported, with no deaths recorded.									
Malawi	Cholera	Protracted 3	26-Dec-25	26-Dec-25	29-Dec-25	11	11	0	0.00%
An outbreak of cholera have been confirmed in Malawi in late December 2025 in Kasungu and Neno districts. As of 29 December 2025, a total of 11 locally acquired cases have been reported across five districts, with no deaths.									
Ongoing Events									
Angola	Cholera	Protracted 3	7-Jan-25	8-Jan-25	28-Dec-25	36 313	937	895	2.50%
From 31 December 2024 to 28 December 2025, Angola reported 36 313 cholera cases, including 895 deaths (CFR 2.5%). Males accounted for 54% (19 549) of reported cases. Among the deaths, 45% (404) occurred outside health facilities and 55% (491) occurred within health facilities.									
Burkina Faso	Humanitarian crisis (Sahel Region)	Protracted 2	1-Jan-19	1-Jan-19	8-Dec-25	4 400 000	-	-	-
Burkina Faso continues to face a protracted humanitarian crisis driven mainly by persistent insecurity, significantly affecting civilian populations. As of July 2025, more than one million internally displaced persons had returned to their areas of origin, while over 167 000 people were newly displaced between January and July 2025. The country also hosts approximately 42 000 refugees and asylum seekers. Ongoing insecurity, compounded by climate shocks and public health emergencies, continues to exacerbate vulnerabilities and disrupt access to essential services. Humanitarian partners estimate that 4.4 million people will require assistance in 2026.									
Burundi	Cholera	Protracted 3	1-Jan-23	1-Jan-25	14-Dec-25	3 056	3 056	13	0.40%
During epidemiological week 50 (week ending 14 December 2025), 18 cases were reported from two districts. From 1 January to 14 December 2025, a cumulative total of 3 056 cases, including 13 deaths (CFR 0.4%), were reported from 14 districts. The most affected districts were Bujumbura Nord (n = 1 096), Cibitoke (n = 577), and Isare (n = 383).									
Burundi	Complex Humanitarian crisis	Ungraded	1-Sep-23	1-Jan-24	16-Dec-25	1 100 000	-	-	-
As of 16 December 2025, Burundi recorded 82 275 arrivals from eastern Democratic Republic of the Congo since 5 December 2025. An estimated 76 015 people are displaced, of whom 16 088 have been relocated to Bweru and Makombe. Women account for 51% of the displaced population.									
Burundi	Mpox	Grade 3	25-Jul-24	25-Jul-24	14-Dec-25	4 599	4 599	1	0.00%
From 25 July 2024 to 14 December 2025, a total of 4 599 confirmed cases of Mpox with one death were reported in Burundi.									
Cameroon	Humanitarian crisis (North-West & South-West)	Protracted 2	1-Oct-16	27-Jun-18	4-Jan-26	1 800 000	-	-	-
In November 2025, the humanitarian situation in Cameroon's North-West and South-West regions remained highly volatile, characterized by persistent insecurity, population displacement, and serious protection concerns. More than 1 700 people were newly displaced, with repeated movements reported across Momo, Bui, Manyu, and Mezam divisions following armed clashes, threats, and fear of reprisals. At least 10 attacks on education were documented, alongside widespread kidnappings, intimidation of community leaders, and incidents of sexual violence, disproportionately affecting women and children. Non-state armed groups continued to impose movement restrictions and establish informal checkpoints, while intensified security operations further limited civilian mobility. Ongoing insecurity and access constraints severely disrupted livelihoods, essential services, and humanitarian operations across the region.									
Cameroon	Humanitarian crisis (Sahel Region)	Protracted 2	31-Dec-13	27-Jun-17	4-Jan-26	476 000	-	-	=
Hundreds of thousands of people have been displaced by ongoing violence along the Cameroon–Nigeria border, where recurrent insecurity, disease outbreaks, and food insecurity continue to drive humanitarian needs. In Cameroon's Far North region, communities have endured the impacts of the Lake Chad Basin crisis for more than a decade, characterized by repeated attacks, population displacement, and severe disruption of livelihoods. An estimated 476 000 people are displaced in Cameroon, contributing to a regional total of 2.9 million displaced persons across the Lake Chad Basin. Affected populations face precarious living conditions, limited access to health care, and chronic food insecurity, exacerbating vulnerabilities among displaced families and host communities.									
Cameroon	Measles	Ungraded	2-Apr-19	1-Jan-25	4-Jan-26	3 052	2 324	-	0.00%
In 2025, Cameroon reported 3 051 suspected measles cases, of which 2 323 were confirmed. Among the confirmed cases, 666 were laboratory confirmed, 1 528 were epidemiologically linked, and 129 were clinically compatible. In epidemiological week 1 of 2026, one additional clinically compatible case was reported, and 25 health districts are currently experiencing active outbreaks.									
Central African Republic	Complex Humanitarian crisis	Protracted 2	11-Dec-13	11-Dec-13	22-Dec-25	2 300 000	-	-	-
The Central African Republic continues to face a complex humanitarian crisis driven by persistent insecurity, weak governance, and regional instability. As of November 2025, an estimated 416 247 internally displaced persons remained in the country, most of whom are hosted by families, placing significant strain on already fragile host communities. Insecurity persists in the southeast and along the border with Sudan, where renewed violence has triggered further displacement. Refugee inflows from Sudan, intercommunal and transhumance-related conflicts, and limited state presence continue to undermine protection and access to basic services. According to the 2026 Humanitarian Response Plan, 2.3 million people require humanitarian assistance, with 1.3 million targeted for multisectoral support amid ongoing fragility and recurrent population movements.									
Central African Republic	Impact of Sudan crisis	Grade 3	1-May-23	1-May-23	4-Jan-26	47 243	-	-	-

As of January 2026, the Central African Republic is hosting 47 243 people fleeing the conflict in Sudan, including 40 332 refugees and 6 911 refugee returnees. Most arrivals, predominantly women and children, enter through Am-Dafock and relocate to Birao, where approximately 27 000 refugees are hosted at the Korsi settlement in Vakaga Prefecture, one of the poorest and most insecure regions of the country. Overall, 39 056 Sudanese refugees are registered, alongside 5 712 internally displaced persons living with host communities. An additional 13 351 refugees are dispersed across hard-to-reach areas, where access to assistance remains constrained by insecurity, logistical challenges, and funding shortfalls.

Central African Republic	Measles	Ungraded		1-Jan-25	28-Dec-25	7 322	544	1	0.00%
--------------------------	---------	----------	--	----------	-----------	-------	-----	---	-------

In December 2025, the Central African Republic reported 559 new suspected measles cases, including eight laboratory-confirmed (IgM-positive) cases in Bangui 1, Bangui 2, and Bangui 3 health districts, and one epidemiologically linked confirmed case in Gamboula Health District. Since the start of 2025, a total of 7 322 suspected cases have been reported, including 544 confirmed cases (242 laboratory confirmed and 302 epidemiologically linked). No new confirmed cases were reported during epidemiological weeks 50 – 52.

Central African Republic	Mpox	Grade 3	3-Mar-22	31-Dec-23	28-Dec-25	164	164	9	5.50%
--------------------------	------	---------	----------	-----------	-----------	-----	-----	---	-------

Between epidemiological weeks 49 and 52 of 2025, 26 suspected mpox cases were reported from seven health districts, including five confirmed cases in Bangui 1 (n = 4) and Berbérati (n = 1). Since the start of the outbreak in epidemiological week 52 of 2023, a total of 164 confirmed cases have been reported. Nine deaths have been reported among confirmed cases (CFR 5.5%). Bangui 1 and Berbérati health districts remain in active outbreak. In December 2025, clade Ib was identified for the first time in the country, in addition to clade Ia. Children under five years of age account for 22% of confirmed cases, with a male-to-female ratio of 1.6.

Chad	Diphtheria	Grade 2	1-Jan-25	1-Jan-25	21-Dec-25	4 802	52	52	1.10%
------	------------	---------	----------	----------	-----------	-------	----	----	-------

From epidemiological week 1 to week 51 of 2025, Chad reported 4 802 suspected diphtheria cases with 52 deaths (CFR 1.1%) across seven of the country's 23 provinces. Of these, 52 were confirmed, including four laboratory confirmed cases and 48 clinically compatible cases.

Chad	Humanitarian crisis (Sahel region)	Protracted 2	11-Feb-22	1-Mar-16	8-Dec-25	4 000 000	-	-	-
------	------------------------------------	--------------	-----------	----------	----------	-----------	---	---	---

Chad continues to face a severe and protracted humanitarian crisis. An estimated 4 million people are expected to require humanitarian assistance in 2026, down from more than 7 million in 2025, but still representing a substantial proportion of the population in need. The crisis is driven by multiple, overlapping shocks, including conflict-related displacement, large influxes of refugees from neighbouring Sudan, worsening food insecurity, climate-related hazards, and limited access to essential services. Large refugee populations and ongoing internal displacement continue to place significant strain on already fragile social systems, while poor infrastructure and insecurity hinder humanitarian access and service delivery.

Chad	Impact of Sudan crisis	Grade 3	15-Apr-23	15-Apr-23	8-Dec-25	904 060	-	-	-
------	------------------------	---------	-----------	-----------	----------	---------	---	---	---

Since the onset of the conflict in Sudan in April 2023, Chad has received more than 904 060 new refugees, primarily from the Darfur region, in addition to 368 421 Chadian returnees recorded by May 2025. These arrivals add to the approximately 400 000 Sudanese refugees already present in the country prior to the crisis. Over the past year, Chad has received more refugees than during the previous two decades combined and now ranks among the five largest refugee-hosting countries per capita globally, placing considerable strain on national services and host communities.

Chad	Poliomyelitis (cVDPV2)	Grade 2	18-Oct-19	1-Jan-25	4-Jan-26	30	30	0	0.00%
------	------------------------	---------	-----------	----------	----------	----	----	---	-------

Between epidemiological weeks 1 and 52 of 2025, Chad reported 26 cases of cVDPV2 and four cases of cVDPV3. The most recent case had paralysis onset on 28 October 2025. In 2024, a total of 39 cVDPV2 cases were reported.

Congo	Mpox	Grade 3	23-May-22	1-Jan-24	21-Dec-25	104	104	1	1.00%
-------	------	---------	-----------	----------	-----------	-----	-----	---	-------

From 1 January to 21 December 2025, 104 mpox laboratory-confirmed cases with one death (CFR 1%) were reported in the Republic of Congo. Sequencing analysis identified clade Ia, clade Ib, and clade IIb.

Democratic Republic of the Congo	Cholera	Protracted 3	16-Jan-15	1-Jan-25	7-Dec-25	67 624	-	2 004	3.00%
----------------------------------	---------	--------------	-----------	----------	----------	--------	---	-------	-------

In epidemiological week 49 of 2025, the Democratic Republic of the Congo reported 1 140 suspected cholera cases, including 22 deaths (CFR 1.9%). Since the beginning of 2025, a cumulative total of 67 624 suspected cases, including 2 004 deaths (CFR 3.0%), have been reported from 237 of the 519 health zones across 21 provinces. As of week 49, 75 health zones in 13 provinces remained in active outbreak. During the preceding four weeks, 73% of reported cases were concentrated in North Kivu, Haut-Lomami, South Kivu, Tanganyika, and Maniema provinces.

Democratic Republic of the Congo	Complex Humanitarian crisis DRC	Grade 3	20-Dec-16	17-Apr-17	4-Jan-26	21 200 000	-	-	-
----------------------------------	---------------------------------	---------	-----------	-----------	----------	------------	---	---	---

See details in article.

Democratic Republic of the Congo	Mpox	Grade 3	30-Mar-19	1-Jan-25	4-Jan-26	22 778	22 778	51	0.20%
----------------------------------	------	---------	-----------	----------	----------	--------	--------	----	-------

From 24 November 2025 to 4 January 2026, the Democratic Republic of the Congo reported 447 confirmed mpox cases with zero deaths. Since the start of 2025, a cumulative total of 22 778 confirmed cases, including 51 deaths (CFR 0.2%), have been reported nationwide. The most affected provinces include Kinshasa, North Kivu, South Kivu, Bas-Uele, Tanganyika, and Tshopo.

Ethiopia	Cholera	Protracted 3	17-Sep-22	1-Jan-25	14-Dec-25	8 444	-	84	1.00%
----------	---------	--------------	-----------	----------	-----------	-------	---	----	-------

From 1 January to 14 December 2025, a total of 8 444 cholera cases with 84 deaths were reported in Ethiopia, with an overall case fatality rate (CFR) of 0.9%.

Ethiopia	Complex Humanitarian crisis-ETH	Protracted 2	4-Nov-20	4-Nov-20	10-Dec-25	21 800 000	-	-	-
----------	---------------------------------	--------------	----------	----------	-----------	------------	---	---	---

Ethiopia continues to face multiple humanitarian emergencies driven by conflict, climate-related shocks, disease outbreaks, and regional instability. As of May 2024, an estimated 3.3 million internally displaced persons were recorded, primarily due to conflict (69%) and drought (17%), with 53% displaced for three years or more. In addition, 2.58 million internally displaced person returnees were identified, mainly in Tigray (38%), Amhara (37%), and Afar (9%). Despite the 2022 Cessation of Hostilities Agreement, insecurity persists in several regions, while recurrent droughts and floods, rising temperatures, and erratic rainfall continue to severely affect pastoralist and agro-pastoralist communities.

Ethiopia	Impact of Sudan crisis	Grade 3	1-May-23	1-May-23	12-Oct-25	76 844	-	-	-
----------	------------------------	---------	----------	----------	-----------	--------	---	---	---

Following the outbreak of the conflict in Sudan on 15 April 2023, Ethiopia has experienced an influx of refugees and returnees from Sudan. As of 12 October 2025, a total of 76 844 new arrivals had been recorded, comprising 45 118 Sudanese refugees and asylum seekers, 21 018 Ethiopian returnees, and 10 708 non-Sudanese refugees and asylum seekers.

Ethiopia	Marburg virus disease	Grade 2	11-Nov-25	14-Nov-25	28-Dec-25	14	14	9	64.30%
----------	-----------------------	---------	-----------	-----------	-----------	----	----	---	--------

As of 28 December 2025, Ethiopia reported 14 confirmed cases of Marburg virus disease. A total of 9 deaths were recorded.

Ethiopia	Measles	Ungraded	13-Apr-17	1-Jan-25	28-Dec-25	8 845	5 572	23	0.30%
----------	---------	----------	-----------	----------	-----------	-------	-------	----	-------

As of 28 December 2025 (Week 52), Ethiopia reported 8 845 suspected cases of measles, including 5 572 confirmed cases and 23 deaths. Active confirmed outbreaks are ongoing in two woredas from the Oromia region.

Ghana	Mpox	Grade 3	4-Mar-25	21-Feb-25	28-Dec-25	944	944	7	0.70%
-------	------	---------	----------	-----------	-----------	-----	-----	---	-------

Ghana reported its first case of mpox on 4 March 2025 in a 29-year-old male from the Greater Accra Region, with symptom onset on 21 February 2025. As of 28 December 2025, a cumulative total of 944 confirmed mpox cases, including seven deaths, had been reported nationwide.

Guinea	Mpox	Grade 3	14-Jun-25	2-Sep-24	4-Jan-26	2 024	2 024	5	0.20%
--------	------	---------	-----------	----------	----------	-------	-------	---	-------

During epidemiological week 1 of 2026, Guinea reported 46 suspected mpox cases, including 33 laboratory-confirmed cases and no associated deaths. From January to December 2025, a total of 3 618 suspected cases were reported, of which 2 024 were confirmed. Five deaths were reported among confirmed cases.

Kenya	Cholera	Protracted 3	1-Oct-25	17-Sep-25	28-Dec-25	296	20	7	2.40%
-------	---------	--------------	----------	-----------	-----------	-----	----	---	-------

As of epidemiological week 52 (ending 28 December 2025), Kenya reported a total of 296 cholera cases, including 20 laboratory-confirmed cases and seven deaths (2.4%). Two new cases were reported during the final week of 2025 from Nyatike Sub-county in Migori County. The highest proportion of cases (n = 133; 45%) occurred among individuals aged 15–44 years. Males accounted for 195 cases (66%) of all reported cases.

Kenya	Measles	Ungraded	29-Jun-22	1-Jan-25	28-Dec-25	895	298	2	0.20%
-------	---------	----------	-----------	----------	-----------	-----	-----	---	-------

During epidemiological week 52 of 2025, Kenya reported 13 new measles cases, including 10 cases from Pokot South Sub-county and three from Naivasha Sub-county. Ten sub-counties are currently experiencing active outbreaks. Since the beginning of 2025, a cumulative total of 895 cases have been reported, including 298 laboratory-confirmed cases and two deaths. Males accounted for 456 cases (51%) of reported cases. Children under five years of age remain the most affected group.

Kenya	Mpox	Grade 3	3-Aug-24	31-Jul-24	28-Dec-25	946	946	13	1.40%
-------	------	---------	----------	-----------	-----------	-----	-----	----	-------

In Week 52 of 2025, four new mpox cases were reported from Busia County (2), Mombasa County (1), and Nairobi County (1). Since the start of the outbreak in week 31 of 2024, a total of 946 confirmed cases, including 13 deaths, were reported.

Liberia	Lassa Fever	Ungraded	3-Mar-22	6-Jan-25	23-Dec-25	34	34	8	23.50%
---------	-------------	----------	----------	----------	-----------	----	----	---	--------

From 18 to 23 December 2025, two new confirmed Lassa fever cases were reported in Liberia. Since January 2025, a total of 215 suspected cases were reported, including 34 confirmed cases and eight deaths among confirmed cases.

Liberia	Mpox	Grade 3	31-Aug-24	31-Aug-24	4-Jan-26	1 518	1 518	8	0.50%
---------	------	---------	-----------	-----------	----------	-------	-------	---	-------

From 1 December 2025 to 4 January 2026, Liberia reported 79 confirmed mpox cases, including two deaths. Since the start of the outbreak in August 2024, a total of 1 518 confirmed cases, including eight deaths, were reported. In 2025 alone, 1 455 confirmed cases and eight deaths were recorded.

Liberia	Yellow fever	Ungraded	18-Nov-25	17-Nov-25	16-Dec-25	1	1	0	0.00%
---------	--------------	----------	-----------	-----------	-----------	---	---	---	-------

On 17 November 2025, a 35-year-old male miner from Mano River Kongo, Porkpa District, Grand Cape Mount County, presented with symptoms compatible with yellow fever. A specimen collected the same day tested presumptive positive at the National Reference Laboratory on 18 November 2025 and was later confirmed as yellow fever at the Institut Pasteur de Dakar on 15 December 2025.

Malawi	Mpox	Grade 3	17-Apr-25	21-Apr-25	4-Jan-26	140	140	1	0.70%
--------	------	---------	-----------	-----------	----------	-----	-----	---	-------

From 16 August 2024 to 23 November 2025, Malawi has confirmed 140 mpox cases, including four cross-border cases and one death. The last case was reported on 23 November 2025.

Mali	Humanitarian crisis (Sahel region)	Protracted 2	11-Sep-17	11-Sep-17	8-Dec-25	5 100 000	-	-	-
------	------------------------------------	--------------	-----------	-----------	----------	-----------	---	---	---

Mali continues to face a complex humanitarian crisis driven by persistent insecurity, climate-related shocks, and structural socio-economic vulnerabilities. Violence and armed clashes, particularly in the north and centre, with increasing spillover to the south and west, continue to drive population displacement, disrupt livelihoods and education, and expose civilians to serious protection risks. The growing use of improvised explosive devices and drones has further restricted humanitarian access and civilian movement. An estimated 5.1 million people, representing approximately 20% of the population, are projected to require humanitarian assistance and protection in 2026, with food insecurity and malnutrition remaining critical concerns.

Mauritania	Diphtheria	Grade 2	17-Sep-25	1-Jan-25	14-Dec-25	1 082	1 082	39	3.60%
------------	------------	---------	-----------	----------	-----------	-------	-------	----	-------

Since epidemiological week 33 of 2025, Mauritania has reported an increase in diphtheria cases, predominantly affecting the Hodh Ech Chargui Region, particularly Bassiknou District, which hosts a large refugee population. Between epidemiological weeks 1 and 50 of 2025, a total of 1 082 cases and 39 deaths were reported nationwide (CFR: 3.6%). Of these, 745 cases (68.9%) with 18 deaths were reported from Hodh Ech Chargui Region.

Mauritania	Rift Valley fever	Grade 2	27-Sep-25	27-Sep-25	30-Nov-25	53	53	15	28.30%
------------	-------------------	---------	-----------	-----------	-----------	----	----	----	--------

See details in article.

Mozambique	Cholera	Protracted 3	30-Nov-24	28-Oct-24	21-Dec-25	5 814	5 814	88	1.50%
------------	---------	--------------	-----------	-----------	-----------	-------	-------	----	-------

As of 21 December 2025, Mozambique reported 5 814 cholera cases, including 88 deaths (CFR 1.5%), across multiple districts in Nampula, Zambezia, Tete, Manica, Cabo Delgado, and Sofala provinces. During epidemiological week 51, 257 new cases and one death were reported, representing a 13% increase compared with week 50. Eight districts remain in active outbreak, with the highest burden reported in Memba, Tete, and Moma. Individuals aged 15 years and above account for the majority of cases (83%), with no significant difference observed by sex.

Mozambique	Humanitarian crisis (Cabo Delgado)	Protracted 2	5-Sep-24	5-Sep-24	4-Jan-26	110 000	-	-	-
------------	------------------------------------	--------------	----------	----------	----------	---------	---	---	---

In late November 2025, attacks by non-state armed groups in Nampula Province triggered large-scale displacement in northern Mozambique, with 82 691 people fleeing Memba District, primarily to Erati and onward to other districts, including parts of Cabo Delgado Province. Children accounted for approximately 67% of those displaced. By 11 December 2025, 12 580 people had returned through government-facilitated movements. The situation was further compounded by a cholera outbreak declared in Erati District on 9 December, as well as growing hostility toward health workers linked to misinformation. The violence has generated acute humanitarian needs in Erati and Memba districts, emerging as a new hotspot not currently prioritized under the 2026 Mozambique Humanitarian Response Plan.

Mozambique	Mpox	Grade 3	11-Jul-25	8-Jul-25	4-Jan-26	93	93	0	0.00%
As of 4 January 2026, Mozambique had reported 1 892 suspected mpox cases, including 93 laboratory-confirmed cases, with zero deaths recorded. Confirmed cases were reported from Niassa (82), Maputo (4), Manica (3), Tete (3), and Cabo Delgado (1) provinces.									
Namibia	Cholera	Protracted 3	27-Nov-25	19-Nov-25	31-Dec-25	80	20	0	0.00%
Namibia declared a cholera outbreak on 27 November 2025 following confirmation of cases in Grootfontein Health District, Otjozondjupa Region. As of 31 December 2025, 80 suspected cases had been reported, including 14 laboratory-confirmed cases and six epidemiologically linked cases.									
Namibia	Measles	Ungraded	14-Sep-25	18-Sep-25	4-Jan-26	863	241	2	0.20%
The outbreak of measles in Opuwo Health District, Kunene Region, Namibia, was officially declared by the Minister of Health through a press release on 15 September 2025. As of 4 January 2026, a total of 241 confirmed cases (187 laboratory-confirmed and 54 epidemiologically linked), including two deaths, had been reported among 863 suspected cases.									
Niger	Diphtheria	Grade 2	28-Aug-23	1-Jan-25	21-Dec-25	2 187	2 187	150	6.90%
Between epidemiological weeks 1 and 51 of 2025, Niger reported a total of 2 187 diphtheria cases, including 150 deaths, resulting in a case fatality ratio of 6.9%.									
Niger	Humanitarian crisis (Sahel region)	Protracted 2	1-Feb-15	1-Feb-15	8-Dec-25	2 600 000	-	-	-
As of 30 September 2025, Niger recorded 938 000 displaced persons, representing a 5.7% increase compared with 2024, primarily in Tillabéri, Diffa, and Tahoua regions. Protection risks remain high, with 1 175 incidents affecting 2 423 victims reported between January and June 2025. Insecurity and flooding have disrupted basic services, leaving 1 097 schools non-functional and affecting 93 676 students, nearly half of whom are girls. Climate-related shocks affected approximately 542 000 people, destroying 54 891 homes and 10 400 hectares of farmland. Food insecurity and malnutrition remain severe, with an estimated 2.2 million people facing acute food insecurity and 412 000 children suffering from severe acute malnutrition. Overall, 2.6 million people are projected to require humanitarian assistance in 2026.									
Nigeria	Cholera	Protracted 3	-	2-Mar-25	21-Dec-25	22 196	-	505	2.30%
Between 1 January and 21 December 2025, Nigeria reported a total of 22 196 suspected cholera cases, including 505 deaths, resulting in a case fatality ratio of 2.4%.									
Nigeria	Dengue fever	Protracted 2	11-Nov-25	13-Sep-25	17-Nov-25	75	8	6	8.00%
In September 2025, Sokoto State, Nigeria, reported cases of a malaria-like illness unresponsive to standard treatment, prompting investigation for dengue fever. As of 17 November 2025, 75 suspected dengue cases, including eight PCR-confirmed cases, had been reported from Sokoto South and Sokoto North LGAs, with six deaths recorded among suspected cases.									
Nigeria	Diphtheria	Grade 2	10-Mar-25	9-May-22	14-Dec-25	43 758	26 502	1 376	3.10%
Between 1 January and 14 December 2025, Nigeria reported 14 857 suspected diphtheria cases, including 14 000 confirmed cases (207 laboratory-confirmed, 780 epidemiologically linked, and 13 013 clinically compatible). A total of 884 deaths were reported during the same period. Transmission remains active in five northern states: Kano, Borno, Yobe, Bauchi, and Katsina. Since 2022, over 43 000 suspected cases have been reported.									
Nigeria	Humanitarian crisis (Sahel region)	Protracted 2	10-Oct-16	10-Oct-16	8-Dec-25	5 900 000	-	-	0.00%
Nigeria continues to face severe humanitarian needs driven by protracted conflict, insecurity, climate-related shocks, and disease outbreaks, particularly in Borno, Adamawa, and Yobe (BAY) states. Sixteen years of conflict have increasingly targeted civilians, with 4 000 deaths recorded in the first eight months of 2025, matching the total reported for all of 2023, and a renewed use of improvised explosive devices at levels last seen in 2017. Climate shocks have further compounded the crisis: floods in 2024 affected more than 5 million people, displaced approximately 1 million, and destroyed 1.3 million hectares of crops, exacerbating food insecurity amid inflation exceeding 20%. In the BAY states, an estimated 4.8 million people require emergency food and nutrition assistance, including 2 million acutely malnourished children. In 2026, approximately 5.9 million people are expected to require humanitarian assistance nationwide, with 42% of locations in Borno classified as facing life-threatening severity levels.									
Nigeria	Lassa Fever	Ungraded	30-Nov-24	1-Jan-25	28-Dec-25	1 157	1 148	215	18.70%
During epidemiological week 52, Nigeria reported 27 confirmed Lassa fever cases from 12 local government areas (LGAs) across five states. From January to December 2025, a total of 1148 confirmed and nine probable cases were reported from 107 LGAs in 22 states. Among confirmed cases, nine deaths were recorded. The majority of confirmed cases (89%) were reported from Ondo, Bauchi, Edo, and Taraba states. The most affected age group was 21–30 years, with a male-to-female ratio of 1:0.8.									
Nigeria	Measles	Ungraded	1-Jan-25	1-Jan-25	30-Nov-25	19 213	19 213	153	0.80%
In November 2025, Nigeria reported 71 suspected measles cases, including one laboratory-confirmed case. From January to November 2025, a total of 26 866 suspected cases were reported, of which 19 213 (71.5%) were confirmed (2 791 laboratory confirmed, 2 013 epidemiologically linked, and 14 409 clinically compatible). A total of 153 deaths were reported among confirmed cases (CFR 0.8%). Five states: Borno (n = 7 968), Zamfara (n = 4 779), Yobe (n = 2 076), Bauchi (n = 1 574), and Kebbi (n = 1 357), accounted for 66.1% of all suspected cases. As of November 2025, only one local government area, Eti Osa in Lagos State, remained in active measles outbreak.									
Nigeria	Mpox	Grade 3	31-Jan-22	1-Jan-24	30-Nov-25	612	612	6	1.00%
Cumulatively, from 1 January 2024 to 8 September 2025, 612 confirmed cases with six deaths were reported.									
Nigeria	Poliomyelitis (cVDPV2)	Grade 2	1-Jun-18	1-Jan-25	4-Jan-26	53	53	0	0.00%
No new cVDPV2 cases were reported during the current reporting week. In 2025, Nigeria recorded a total of 53 cVDPV2 cases, with the most recent case having a paralysis onset date of 25 October 2025. The cumulative total for 2024 stands at 98 cases. By comparison, 87 cases were reported in 2023, and 48 cases in 2022.									
Rwanda	Mpox	Grade 3	24-Jul-24	24-Jul-24	4-Jan-26	128	128	0	0.00%
From 24 July 2024 to 4 January 2026, Rwanda has reported 128 confirmed cases of mpox with zero deaths. The most recent case had symptom onset on 12 October 2025.									
Senegal	Dengue fever	Protracted 2	23-Sep-25	1-Jan-25	4-Jan-26	6 671	6 671	3	0.00%
During epidemiological week 1 of 2026, three dengue cases were reported from Ndoffane District in Kaolack Region, Senegal. In 2025, the country reported 6,668 laboratory-confirmed dengue cases, including three deaths, across 68 districts in 14 regions, with the highest burden in Kaolack, Fatick, Saint-Louis, and Dakar. The median age of cases was 23 years, and just over half (53.3%) occurred among males.									
Senegal	Measles	Ungraded	4-Jul-22	1-Jan-25	4-Jan-26	123	123	0	0.00%
From January to December 2025, Senegal reported 123 laboratory-confirmed measles cases, with males accounting for 56% of cases (n = 70). Cases were reported from 40 health districts across 12 regions, with the highest numbers recorded in Louga (n = 39), Kédougou (n = 28), Thiès (n = 13), and Kaffrine (n = 8) regions. The median age of cases was 7 years (range: 4–54 years). Among children aged 9 months to 5 years, 23 of 39 cases (59%) were unvaccinated. No cases were reported during epidemiological week 1 of 2026; however, two districts remain in active outbreak.									
Senegal	Rift Valley fever	Grade 2	26-Sep-25	13-Sep-25	4-Jan-26	561	561	31	5.53%

See details in article.									
Sierra Leone	Mpox	Grade 3	11-Jan-25	9-Jan-25	4-Dec-25	5 442	5 442	60	1.10%
From 9 January to 4 December 2025, 5 442 mpox confirmed cases with 60 deaths (CFR 1.1%) were reported from all the 16 districts of Sierra Leone.									
Sierra Leone	Cutaneous Ulcer	Ungraded	24-Nov-25	17-Nov-25	15-Dec-25	303	-	0	0.00%
Between 17 November and 12 December 2025, 303 cases of a skin disease of unknown aetiology were reported from Safroko Limba Chiefdom, Bombali District. Most cases occurred among males, and investigations ruled out cutaneous anthrax, with findings suggesting <i>Haemophilus ducreyi</i> as the most likely cause.									
South Africa	Diphtheria	Grade 2	19-Dec-24	1-Jan-24	4-Jan-26	90	88	19	21.60%
Between 1 January 2024 and 4 January 2026, South Africa reported a total of 88 confirmed and two probable cases of respiratory diphtheria. The majority of confirmed cases were reported from the Western Cape (n = 66). The median age of confirmed cases was 25 years (range: 2–55 years), with 69% (62/90) occurring among individuals aged 18 years and older. The overall case fatality ratio among confirmed and probable cases was 21.6%.									
South Africa	Measles	Ungraded	16-Jun-25	19-Jun-25	28-Dec-25	2 756	2 756	0	0.00%
During epidemiological week 52 of 2025, new laboratory-confirmed measles cases were reported from Free State (n = 2), Limpopo (n = 8), Mpumalanga (n = 10), Northern Cape (n = 6), and Western Cape (n = 68). Since the beginning of 2025, a cumulative total of 2 756 laboratory-confirmed measles cases have been reported nationwide.									
South Africa	Mpox	Grade 3	30-Oct-25	1-Jan-25	4-Jan-26	15	15	0	0.00%
Between weeks 49 and 51 of 2025, three new mpox cases were reported in South Africa, following more than five consecutive weeks with no reported cases. In total, 15 laboratory-confirmed mpox cases were reported in 2025 from Gauteng (8), KwaZulu-Natal (5), North West (1), and Western Cape (1) provinces.									
South Sudan	Anthrax	Ungraded	1-Aug-24	1-Jan-24	21-Dec-25	377	1	5	1.30%
In 2025, South Sudan reported a total of 216 human anthrax cases, including two deaths, all originating from Western Bahr el Ghazal State (n = 177) and Warrap State (n = 39). Since the outbreak began in 2024, a cumulative total of 377 cases have been reported, including five deaths (CFR 1.3%).									
South Sudan	Cholera	Protracted 3	11-Oct-24	28-Sep-24	28-Dec-25	97 257	12 593	1 581	1.60%
As of 28 December 2025, South Sudan has reported a total of 97 257 suspected cases since the onset of the outbreak on 28 September 2024.									
South Sudan	Complex Humanitarian crisis -SS	Protracted 3	15-Aug-16	15-Aug-16	13-Jan-26	10 000 000	-	-	-
South Sudan continues to face a severe and protracted humanitarian crisis driven by conflict, climate-related shocks, disease outbreaks, and economic fragility. In 2026, more than 10 million people, nearly two-thirds of the population, are projected to require humanitarian assistance. Recurrent flooding and drought have affected over 1 million people annually since 2019, with an estimated 1.3 million people impacted and 375 000 displaced in 2025 alone. Ongoing insecurity and intercommunal violence continue to drive displacement and heighten protection risks. Public health conditions remain critical, marked by the country's largest cholera outbreak, alongside concurrent hepatitis E, mpox, and malaria outbreaks.									
South Sudan	Impact of Sudan crisis	Grade 3	15-Apr-23	1-May-23	4-Jan-26	1 300 000	-	-	-
The conflict in Sudan, now in its third year, continues to place severe strain on South Sudan's humanitarian system. By the end of November 2025, nearly 1.3 million refugees and returnees had entered South Sudan since April 2023, with an additional 380 000 arrivals projected by the end of 2026. This influx has overwhelmed host communities, particularly in Renk and Maban, where basic services are operating at 300–400% of capacity. Rising food and fuel prices, reduced humanitarian funding, and competition over land and resources have further heightened vulnerabilities and tensions. The crisis has also contributed to cross-border disease transmission, with South Sudan's ongoing cholera outbreak originating in Sudan, alongside increased risks of measles and hepatitis E.									
South Sudan	Mpox	Grade 3	7-Feb-25	7-Feb-25	4-Jan-26	38	38	0	0.00%
In 2025, South Sudan reported a total of 38 laboratory-confirmed mpox cases, with zero deaths. The most recent case was reported on 7 December 2025.									
Tanzania, United Republic of	Cholera	Protracted 3	3-Oct-23	1-Jan-25	14-Dec-25	4 261	-	58	1.40%
In 2025, as of 14 December, the United Republic of Tanzania reported a 4 261 cholera cases, including 58 deaths (CFR 1.4%). During the preceding 28 days, 21 new cases were reported.									
Tanzania, United Republic of	Mpox	Grade 3	10-Mar-25	10-Mar-25	4-Jan-26	268	268	0	0.00%
From 25 February to 31 December 2025, the United Republic of Tanzania reported 268 laboratory-confirmed mpox cases, with no deaths recorded (CFR 0%). Eighteen cases were reported during the past six weeks. The most recent case was reported on 28 December 2025.									
Togo	Measles	Ungraded	13-Oct-24	13-Oct-24	4-Jan-26	1 130	660	0	0.00%
Togo has been experiencing a measles outbreak since epidemiological week 41 of 2024. As of 4 January 2026, a total of 1130 suspected cases were reported from affected districts, including 660 laboratory-confirmed cases, with no deaths. During epidemiological week 1 of 2026, 73 new suspected cases were recorded. The median age of cases was 6 years (range 1 month–55 years).									
Togo	Mpox	Grade 3	12-May-25	2-May-25	4-Jan-26	88	88	0	0.00%
In 2025, Togo reported 88 laboratory-confirmed mpox cases, with the last case notified on 9 November 2025.									
Uganda	Mpox	Grade 3	26-Jul-24	29-Jul-24	4-Jan-26	8 041	8 041	50	0.60%
As of 4 January 2026 2025, Uganda has reported 8 041 confirmed cases of mpox with 50 deaths (CFR 0.6%) from 120 districts affected across the country.									
Zambia	Cholera	Protracted 3	30-Dec-24	25-Dec-24	14-Dec-25	1 094	-	18	1.60%
As of 14 December 2025, Zambia reported a cumulative total of 1 094 cholera cases, including 18 deaths (CFR 1.6%). During the preceding 28 days, 44 cases with zero deaths were recorded.									
Zambia	Mpox	Grade 3	8-Oct-24	8-Oct-24	4-Jan-26	367	367	3	0.80%
Since the outbreak began in October 2024, Zambia has reported a total of 267 laboratory-confirmed mpox cases, including three deaths, as of 4 January 2026. Of these, 265 cases and all three deaths occurred in 2025. The most recent case was reported on 14 December 2025.									

Note: There are 25 additional ongoing events not included in this table. These events will be included subsequently when additional details are available.

†Grading is an internal WHO process, based on the Emergency Response Framework. For further information, please see the Emergency Response Framework: <http://www.who.int/hac/about/erf/en/>.

Data are taken from the most recently available situation reports sent to WHO AFRO. Numbers are subject to change as the situations are dynamic.

Click [here](#) to subscribe and receive the weekly edition of the Bulletin on Outbreaks and Other Emergencies

© WHO Regional Office for Africa

This is not an official publication of the World Health Organization.

Correspondence on this publication may be directed to:
Health Emergency Intelligence, Surveillance, and Laboratory Programme
WHO Emergency Preparedness and Response Cluster
WHO Regional Office for Africa
P O Box. 06 Cité du Djoué, Brazzaville, Congo
Email: afrooutbreak@who.int

Requests for permission to reproduce or translate this publication – whether for sale or for non-commercial distribution – should be sent to the same address.

WCO Contributors

- DIALLO, Amadou Mouctar (DRC)
- LUBOYA, Henri Mbiya Ngandu (DRC)
- PALLAWO, Raymond (Mauritania)
- OKOU, Hilde Fonse Bisso (Senegal)
- BA, Ibrahim Oumar (Senegal)

Design and Publication

- MLANDA, Tamayi
- DONKOR, Gianni-Ferrari

AFRO Editorial Team

WILLIAMS, George Sie
KAVOGA, Freddy Kambale
KIBANGOU, Emerencienne
SARR, Mohamet Lamine Dethie
WAMBA, Gaston
OLAYINKA, Adebola T.
OTSHUDIEMA, John
MUWANGUZI, Esther

Editorial Advisory Group

Dr Marie Roseline Darnycka Belizaire,
Regional Emergency Director

Dr Etien Koua, Program Area Manager,
EIS/EPR/AFRO

Dr Dick Chamla, Program Area Manager,
HEP/EPR/AFRO

Dr Otim Patrick Ramadan, Program Area
Manager, EMR/EPR/AFRO

Data sources

Data and information is provided by Member States through WHO Country Offices via regular situation reports, teleconferences and email exchanges. Situations are evolving and dynamic therefore numbers stated are subject to change.